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U.S. Department of Health and Human Services
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Submitted via www.regulations.gov

RE: Request for Information; Health Technology Ecosystem [CMS-0042-NC]

Dear Drs. Oz and Keane:

Valer Health applauds CMS and ASTP/ONC's interest in using technology to transform the healthcare ecosystem. We are supporting this mission by helping healthcare providers streamline prior authorizations through automated workflows. Valer offers a tech solution that will allow providers to take advantage of the CMS Interoperability and Prior Authorization final rule, which mandates APIs for prior authorization purposes.

For over 13 years, we have been mapping prior authorization submission and verification workflows between providers and payers to build a network that supports the electronic exchange of prior authorization data and drives automation. We have learned that the absence of adopted standards is only part of the problem. There is a foundational need to untangle and translate the complex prior authorization workflows that have developed between payers and providers. Over the years, each stakeholder has created their own set of workflows, business logic, and workarounds -- reflecting critical gaps in current systems. Prior authorization requires a universal translation network to unlock real communication, automation, and efficiency.

The current prior authorization landscape also requires flexibility. Any effective solution must accommodate a wide range of workflows and business process optimizations and adapt as those evolve. Existing models that suggest there is only one way to do this fail to reflect the operational realities payers and providers face in managing prior authorizations.

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Overall, Valer urges CMS to **create policies that do not solely rely on the more prescriptive HL7 FHIR**, and instead **allow for alternative, market-based solutions** to evolve in the marketplace that meet the stated goals of driving real automation, efficiency, and accountability into prior authorization workflows.

Valer offers its expertise to make the following specific recommendations about two RFI questions:

- PR-3: How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)?
- PA-2: How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

Making Electronic Health Record (EHR) Data More Accessible

Data collected by electronic medical record systems must be accessible to use and exchange by health system stakeholders, but there are multiple challenges and technical barriers to address. The reason why providers and payers currently use fax and portal automation to accomplish prior authorization exchanges is because it works. Any future model for exchange must support the shift from today's generally functional exchange system to a new, more integrated future state.

Health plans must develop FHIR APIs by January 1, 2027, according to the CMS Interoperability and Prior Authorization final rule, but FHIR APIs are not equipped to handle the practical workflows, inherent limitations, and business rules involved in prior authorization. Without changes, there will be a lengthy period of transition to incorporate a mix of legacy and API workflows, adding to the already complex prior authorization process.

There are several challenges associated with FHIR APIs:

- **FHIR APIs can take years to implement**. Innovative workflow arrangements are at the mercy of standards processes that take months or years to adapt. Moreover, providers must incorporate version updates into their planning to incorporate new features.
- **There is a lack of data fidelity between EHR workflows and payer requirements.** To fill the current data gaps, humans must manually enter and translate the data. This, of course, leads to errors, delays, and denials. At the same time, DaVinci FHIR APIs do not provide for a UI/UX that can handle data gaps, or errors and exceptions caused by the lack of data fidelity, so humans must manually address these problems.
- **Payer and provider workflows are misaligned.** With the push for DaVinci FHIR APIs, practical implementation of complex prior authorization workflows will be difficult both on the EHR and payer side. Much of today's business rules and logic related to prior authorizations are embedded within payer web portals. Existing payer UI/UX allows for the implementation of complex health plan business logic, contractual rules, carve-outs, etc. The DaVinci FHIR APIs do not reflect the complex business logic (e.g., step therapy, in-network providers, etc.) and contractual complexities (e.g., health plan lines of business, secondary coverage, employer-specific contracts, carve-outs, etc) that are part of the prior authorization process.
- **FHIR APIs are ill-equipped to handle health plans' compliance requirements from various oversight entities (e.g., CMS, NCQA, URAC).** These rules necessitate logging and auditing at a

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granular level which is limited with FHIR APIs. FHIR APIs, while efficient at standardized methods to retrieve, create, and update discrete data exchange, lack the granularity and process flows often required with prior authorization workflows (user, step therapy, clinical info requested/provided, peer-to-peer, etc).

Valer is well-positioned to help CMS address these challenges.

Valer knows the players and service lines. We are familiar with a broad footprint of automated payer web portal submission and verification across national TPAs (e.g., Availity, Carelon, eviCore, Cohere), national payer web portals (e.g., UHC, Aetna), and numerous local/regional health plans. We also integrate across a number of provider EHR systems, including Epic, Cerner/Oracle, athenahealth, Veradigm, and Altera. This allows us to understand the nuances and gaps in existing EHR prior authorization workflows to provide solutions. Moreover, Valer provides an all-service line footprint (e.g., diagnostic imaging, surgery, procedures, specialty medications, infusions, PT/OT/Rehab, E&M, NOA, etc) and across most specialties, with the exception being formulary-based pharmacy benefit medications that are excluded from the CMS rule.

Valer is participating in a DaVinci FHIR demo. We are currently in the process of implementing DaVinci FHIR API CRD, DTR, PAS bundles in a demonstration project. In parallel, we are implementing proprietary payer and drawing on our expertise with payer web portal automation to further act as a key bridge across workflows, further promoting automation and efficiency. In our DaVinci FHIR work, we have identified many practical implementation considerations reflective of today's existing prior authorization provider and payer workflows (e.g., absence/timing of CPT/HCPCS code availability within orders to trigger CRD, DTR; physician/provider requirement to drop order, then respond to a series of clinical survey questions – most prior authorization workflows are handled by patient access administrative staff).

We feel strongly that a market-driven approach, one that drives value for both provider and payer stakeholders, should be considered as part of the framework for further automation of prior authorization workflows in parallel or in conjunction with the DaVinci FHIR efforts.

How CMS Can Encourage Payers to Accelerate APIs Implementation

To address the challenges with FHIR APIs listed above, CMS should allow for **market-based implementations** that facilitate prior authorization workflow exchanges between payers and providers to **stand alongside the existing standards-based efforts**. There are existing implementations of market-based prior authorization solutions that are already driving real automation and value for providers that should be allowed to inform, support, and drive automation forward rather than a prescriptive standards-based model that does not take into consideration the existing reality and challenges of prior authorization workflows.

This co-existence model is the best way to ensure that the standards can be interoperable with current technologies in place, allow for the emergence of new technologies to prove their merit, and inform future standards development work with solutions strategies succeed in the market based on delivered benefits.

To address the challenges currently associated with FHIR APIs, CMS could:

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- Create a unifying source of truth for demographic information/identity management (eg, id.me, login.gov, CLEAR) for patients/members and referring/rendering providers across both provider and payer systems to reduce identity mismatches that lead to errors in prior authorization workflows. Consideration should be given for a standardized, consistent provider identity management process utilized during the credentialing process across all payer entities.
- Mandate the standardized mapping between provider EHRs and payer web portal/UM systems. Current standardization efforts rely on USCDI, SNOMED, LOINC, RxNorm for clinical data documentation; prior authorizations currently utilize ICD-10, CPT, HCPCS codes which would require crosswalks/mappings to existing guidelines that accurately map between EHRs and payer UM systems.
- Require consistent data integrity and availability of CPT/HCPCS/ICD-10 codes in EHR workflows (e.g., Cerner/Oracle instances frequently do not provide CPT codes or may only provide 1 code)

Valer's technology can bridge existing processes and future state standards-based interoperability and facilitate the rapid adoption of the interfacing systems. We establish bi-directional data and document exchanges between payer and providers using any means possible. In practice, this means leveraging direct database connections, flat file feeds, custom APIs, X12 278, and a range of both proprietary and standard interface models. HL7 has been the least useful of all interfacing options due to its rigid data models.

Accordingly, we urge CMS to create policies that allow alternative, market-based solutions to drive automation, efficiency, and accountability into prior authorization workflows.

We look forward to assisting the administration by providing relevant, real-world authorization workflow expertise and perspectives. Additionally, Valer can share perspectives on current health plan behaviors with respect to prior authorization response times and approval/denials to support CMS Interoperability and Final Rule regulations.

Thank you for consideration of Valer's feedback and recommendations. Our vision, approach, and the lessons we've learned from over a decade's worth of experience painstakingly mapping prior authorization submission and verification workflows between providers and payers lay the necessary infrastructure to bridge the current workflow divide and make progress to #FixPriorAuth.

Please contact Dr. Steve Kim at steve@valer.health with any questions or to discuss our recommendations further.

Regards,

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